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The Ethics of Clinical Trials Abroad:

A Reproduction of Historical Imperialism in Developing Nations

In 1997, the United States government funded clinical trials in sub-Saharan Africa and Southeast Asia to test a short-course AZT regimen for reducing mother-to-child HIV transmission — trials that would have been categorically impermissible on American soil. Researchers assigned thousands of HIV-positive pregnant women to placebo control groups despite the existence of a proven treatment, reasoning that local standards of care made such a design ethically defensible. The ensuing controversy reignited one of bioethics' most contentious debates: when, if ever, is it permissible to conduct placebo control trials on populations in developing nations? This question sits at the uncomfortable intersection of medical science, global justice, and postcolonial politics. On one side stand researchers and pharmaceutical interests who argue that placebo control trials are scientifically superior, more efficient, and ultimately beneficial to the communities they study. On the other side stand critics who see in these trials the familiar architecture of exploitation — wealthy nations extracting value from vulnerable populations while offering little in return. I argue that placebo control trials conducted in developing nations, as currently practiced, cannot be ethically justified. Informed consent in these contexts is structurally compromised, utilitarian defenses of such trials collapse under scrutiny, and the broader pattern of conducting research that would be illegal at home on foreign

populations constitutes not a neutral scientific enterprise, but a reproduction of historical imperialism dressed in the language of medicine.

The two most common methodologies for clinical trial research of novel drugs are active control trials (ACTs) and placebo control trials (PCTs). In both ACTs and PCTs, participants in the experimental arm are given the treatment being tested and participants are randomly selected to either the experimental arm or the control arm. In the experimental arm, participants are given the novel drug. The primary difference between ACTs and PCTs is treatment given to the control arm. In the control arm for ACTs, participants are given the best known treatment, while in PCTs they are administered a placebo, essentially not treating the participant. This may seem intuitively immoral, but advocates for PCTs justify their utility with the various advantages PCTs have over ACTs. PCTs tend to be significantly more efficient, both in time and resources, than ACTs. This is due to PCTs requiring less treatment and fewer personnel in researchers and participants than ACTs. Furthermore, many researchers will argue that PCTs are more scientifically informative than ACTs or alternative historical comparisons. These advantages support instrumental reasons for deploying PCTs, especially during pandemics with novel diseases.

Proponents of placebo control trials will appeal to a variety of arguments in their defense, with a majority of them being motivated by an ideology of consequentialism. The first of these arguments is an appeal to scientific accelerationism – that scientific progress must be left largely unhindered because of all of the benefits it bestows upon humanity. Advocates of this view judge that the efficiency and efficacy of PCTs justify their use on the moral basis that the quicker medical progress can be made, the more lives can be saved or improved because of it. This is the utilitarian foundation underlying the tradeoff of present harm in the trials for multitudes of future

wellbeing as a result of the research. The calculus is straightforward: every additional day without the existence of a novel treatment is an additional day that lives are lost to disease. And for the participants in the trials, they would have counterfactually suffered the disease with no treatment regardless. This is the next consequentialist argument, which embodies the utilitarian framework more wholly. It posits that intervention should be understood in terms of local standards of care, and thus PCTs do no harm in regions where the local standard of care is poor or nonexistent for the targeted ailment. In these localities, the dominant strategy of diseased individuals is to volunteer for the trials. The expected utility of the possibility of treatment in this case is greater than the alternative option of guaranteed non-treatment. It is therefore in the best interest of individuals to participate. Furthermore, this logic extends to justifying the deployment of PCTs even when treatment already exists. Pharmaceutical corporations or state-funded research groups may prefer conducting no trials at all rather than shoulder the costs of ACTs, thus creating a false dichotomy of choice between PCTs or nothing. In a similar utilitarian line of reasoning as before, conducting PCTs is viewed as infinitely more beneficial compared to the option of no trials, no individuals treated, and no scientific progress achieved. Alongside the utilitarian arguments in favor of PCTs, there are numerous value-based arguments for their use, especially in reference to developing and low-wealth nations. Proponents view volunteer participation in PCTs as a mutualistic and valid agreement between two consenting parties. In the neoliberal worldview, this exchange is validated under principles governing the free market. To monitor and regulate such interactions at too fine of a grain would reek of loathed paternalism. The values of liberty, individualism, and autonomy would be violated by prohibiting participation.

The utilitarian mode of thinking must be wholly and decisively rejected. Game theory cannot operate perfectly in the real world. Human wellbeing cannot be quantified and thus commodified, and the reduction of lives to numbers has real-world consequences. In the case of clinical trials, quantification misses key aspects of wellbeing outside of the scope of the trials themselves. Equating treatment to wellbeing fails to account for the wellness of participants after the study has concluded, their mental wellbeing, and structural issues within their society. On the first point, the standard of care offered in the trial becomes normalized to the participants who are left stranded once the trial concludes by the inaccessibility of the quality of care they had received. This can reinforce and even worsen preexisting conditions that were alleviated by medical care. Furthermore, western nations granting donations to low-income nations creates negative externalities contradictory to the so-called altruism of the research mission. Low-wealth nations become dependent on foreign aid, and the governments become complacent in remedying the structural and institutional shortcomings that cause the need for aid in the first place. There are even cases so extreme that roving gangs form to compete for access to aid deliveries. In this way, outside aid directed by utilitarian motivations perpetuate poverty.

Unfortunately for the libertarian defenders of PCTs, the consent of participants in developing nations is underinformed and less than freely given. The requirements for informed consent that the researchers' home nations employ do not translate into the same concepts in the new context of a foreign country. Reading terms and conditions cannot carry the same weight in a township with high illiteracy rates. Informed consent can only be gained by an explicit, understandable, communicable methodology relevant to the particular community's linguistic, cultural, and ethical norms. Even after attaining truly informed consent, the morality of PCTs is nebulous. Participants operate on the assumption that they are being treated, even if they are in

the control arm. This deception arises from the overall dehumanizing treatment of participants as means to an end. An objection to the labeling of clinical trials in developing nations as exploitative may claim that researchers are not taking unjust advantage of the participants, but rather taking advantage of an unjust situation. What this objector cannot deny, however, is that the researchers are taking advantage of the vulnerabilities of participants. After all, participants are not the only ones who stand to benefit from the execution of the trials, and although the researchers themselves are not culpable for the unjust situation, the wealthy developed nations they represent are culpable for global poverty, and the researchers further perpetuate this injustice. The fact that this risky method of research is illegal or heavily regulated in the researchers' home countries points to a moral contradiction on behalf of the researchers. The participants in developing countries would likely not participate if their local standard of care was of comparable quality to developed countries. The methods that poverty makes available should not make them morally permissible. The looming emphasis of costs and the major role that money plays in this ethical discussion gestures to the possibility that researchers choose developing countries for minimizing risks and costs and then justify it disingenuously by ethical standards. The situation is retroactively ethics-washed.

Clinical trials in developing nations and low-wealth communities have a history of exploitation – PCTs continue this trend as a reproduction of imperialism. Data is created and harvested from vulnerable populations in poor countries, paralleling the historical theft of natural resources by Western powers. Developing nations become risk-tolerant laboratories for experiments by pharmaceutical companies and state actors. Participants are cheap and willing, regulation is relaxed, risk of litigation is low, and there are far fewer ethical reviews of research conducted. These practices echo the human experiments done on marginalized populations and

prisoners of war from Imperial Japan and Nazi Germany. The populations of developing nations are similarly dehumanized and used as a means to a scientific end. Rather than subjugation by war or fascism, developing countries suffer economic subjugation at the hands of capitalism and historical contexts of colonialism. When profits are the end goal, perverse incentives are an inevitability. Cheap clinical trials in these nations qua human laboratories encourage keeping developing nations impoverished to provide consistent access to human lab rats. This dynamic is especially insidious in treatments that are then only made available in the developed nations conducting the research. The deception is then proliferated by narratives pushed to hide the exploitative nature of this practice. Narratives of medical data as a "natural resource" invisibilizes the labor of humans who undergo invasive procedures to create such data.

I am not against conducting medical research in developing countries, nor am I against utilizing PCTs when no alternative treatment exists. Decentering the historical baseline of "white men" in medical research is an important step to making healthcare more accessible and effective for everyone. Furthermore, I don't think that the "standard of care" should be normative and universal – trials should treat participants with methods accessible to them after the trial's conclusion. Implementing a local standard of care in a trial should not, however, be justified as a cost-cutting measure. This is ill-intentioned and thus immoral. The impetus also falls on researchers to have a plan for their participants post-trial, and effective treatments must be made available to the community who offered their labor in testing it. If this is not possible, the trial cannot be ethically justified. Local governments must also be held to a higher standard of protecting their citizens by implementing ethical reviews and conducting independent party audits in a transparent manner. Finally, the purpose of medical research must be respected in

every trial conducted. Research is done for the benefit of humanity, and thus the humanity of participants must be prioritized first and foremost.

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